WEAVE

CENTRE FOR INTEGRATIVE PSYCHIATRY

WEAVE PSYCHOTHERAPY — VOL. 11

Disorder-Specific Therapy Map

Master Cross-Reference — Which Therapy for Which Disorder

Depression · Anxiety · OCD · PTSD · Personality Disorders · Eating Disorders · Psychosis · Bipolar

Dr. Wilfred D'souza

MD Psychiatry

wilfred.desouza1996@gmail.com

Contents

01 Deep Study

02 Clinical Quick Reference

01

Deep Study

Disorder-Specific Therapy Map — Weave Psychotherapy
Vol. 11

D1: Disorder-Specific Therapy Map — Deep Study

Table of Contents

1. How to Read This Map
2. Depressive Disorders
3. Anxiety Disorders
4. OCD and Related Disorders
5. Trauma and Stressor-Related Disorders
6. Personality Disorders
7. Eating Disorders
8. Substance Use Disorders
9. Psychotic Disorders
10. Bipolar Disorder
11. Child and Adolescent Disorders
12. Couples and Relational Problems

1. HOW TO READ THIS MAP

1.1 Evidence Levels Used in This Volume

This volume synthesises guideline recommendations from three principal sources. When recommendations diverge, the most conservative (highest evidence threshold) position is taken.

Source	Grading System	What "First-Line" Means
NICE (UK)	Based on systematic reviews; recommendations rated by committee consensus after evidence review	Recommended as initial treatment; supported by at least one high-quality RCT or meta-analysis
APA (US)	Level I (RCT), Level II (controlled trial), Level III (open trial/case series)	Substantial clinical confidence from Level I evidence
Cochrane	Systematic review with GRADE framework (high, moderate, low, very low certainty)	Moderate-to-high certainty evidence of clinically meaningful effect

EXAM PEARL

NICE guidelines are the most commonly examined guideline system in UK and Commonwealth psychiatry exams. They are updated regularly and freely available. APA guidelines are older and sometimes lag behind evidence — always check the publication date.

1.2 How to Interpret Effect Sizes

Cohen's d	NNT (approx.)	Interpretation
0.2	~16	Small effect
0.5	~6	Medium effect
0.8	~4	Large effect
1.0+	~3	Very large effect

EXAM PEARL

In psychotherapy research, a Cohen's d of 0.5 is clinically meaningful. Cuijpers et al. (2019) found an overall effect size of d = 0.72 for psychotherapy vs. control for depression — this is comparable to antidepressant effect sizes when both are compared against pill placebo.

1.3 NNT in Psychotherapy

The Number Needed to Treat (NNT) tells you how many patients must receive the intervention for one additional patient to benefit compared to control. An NNT of 4–6 is considered good for psychotherapy. For context: NNT for antidepressants in moderate-severe depression is approximately 7 (Cipriani et al., 2018 Lancet network meta-analysis).

CLINICAL ANCHOR

When an examiner asks "Is psychotherapy as effective as medication?" the evidence-based answer for depression is: broadly equivalent in mild-moderate depression, medication may have faster onset in severe depression, but combination is superior to either alone (Cuijpers et al., 2020).

2. DEPRESSIVE DISORDERS

2.1 Major Depressive Disorder (MDD)

NICE CG90/NG222 Stepped Care

Step	Severity	Recommended Intervention
1	Subthreshold / mild	Active monitoring, guided self-help, computerised CBT, structured physical activity
2	Mild-moderate	Low-intensity CBT (guided self-help, group CBT), behavioural activation, individual CBT
3	Moderate-severe	CBT (16–20 sessions), IPT (16 sessions), behavioural activation, or antidepressant medication
4	Severe / complex / treatment-resistant	Combined medication + high-intensity psychological therapy, specialist referral

First-Line Psychotherapies

Cognitive Behaviour Therapy (CBT)

- The most extensively studied therapy for depression. Over 200 RCTs. Cuijpers et al. (2019): d = 0.71 vs. control.
- Beck's cognitive model: negative automatic thoughts arising from depressive core beliefs (helplessness, unlovability, worthlessness) maintain depressed mood. Treatment targets cognitive distortions and behavioural withdrawal.
- Standard protocol: 16–20 sessions, weekly, structured (mood check, agenda, homework review, skill work, new homework, summary, feedback).

- Key techniques: thought records, behavioural experiments, activity scheduling, graded task assignment, cognitive restructuring.

> EXAM PEARL
>
> The landmark study is Rush et al. (1977) — the first RCT showing cognitive therapy was as effective as imipramine for depression. The NIMH Treatment of Depression Collaborative Research Program (Elkin et al., 1989) is the other must-know trial: CBT = IPT = imipramine for mild-moderate depression; imipramine superior only in severe depression.

Interpersonal Therapy (IPT)

- Developed by Klerman and Weissman (1984). Focuses on four interpersonal problem areas: grief, role disputes, role transitions, and interpersonal deficits.
- Does NOT target cognitions directly. The theory: depression occurs in an interpersonal context, and resolving interpersonal problems resolves depression.
- 12–16 sessions, time-limited. NICE recommends it as equivalent to CBT for moderate-severe depression.
- Evidence base: Elkin et al. (1989), Cuijpers et al. (2011) meta-analysis showing equivalence with CBT.

Behavioural Activation (BA)

- Based on Lewinsohn's model: depression is maintained by reduced positive reinforcement from the environment due to behavioural withdrawal.
- Dimidjian et al. (2006): BA was as effective as antidepressant medication and superior to CBT for severe depression in the acute phase.
- NICE recommends BA as a first-line treatment equivalent to CBT. Advantages: simpler to train therapists, can be delivered by non-specialists.
- Key techniques: activity monitoring, activity scheduling, graded task assignment, avoidance hierarchy targeting.

CLINICAL ANCHOR

BA is increasingly favoured in resource-limited settings (including India) because it requires fewer therapist training hours than CBT and can be delivered by lay health workers. The MANAS trial (Patel et al., 2010, Lancet) demonstrated that collaborative stepped care including BA delivered by lay counsellors was effective for common mental disorders in Goa.

Second-Line Psychotherapies

Short-Term Psychodynamic Psychotherapy (STPP)

- Leichsenring et al. (2004) meta-analysis: d = 1.17 (pre-post) for depression. Comparable to CBT in head-to-head trials.
- Gabbard notes indications for LTPP include depression with characterological substrate, especially when personality factors maintain the depressive pattern.
- Techniques: transference interpretation, defence analysis, working through of unconscious conflict maintaining depressive position.

Mindfulness-Based Cognitive Therapy (MBCT)

- Segal, Williams, and Teasdale (2002). Specifically designed for relapse prevention in recurrent depression (3+ prior episodes).
- NICE recommends MBCT as a first-line relapse prevention strategy. NNT = 4 for preventing relapse in those with 3+ prior episodes (Kuyken et al., 2016).
- 8–week group programme combining mindfulness meditation with cognitive therapy elements. NOT recommended for acute depression.

EXAM PEARL

MBCT reduces relapse risk by approximately 43% compared to treatment as usual (Piet & Hougaard, 2011). It is most effective for patients with 3+ prior episodes and those with childhood adversity (Williams et al., 2014). It is NOT first-line for acute depression — it is a relapse prevention intervention.

Persistent Depressive Disorder (Dysthymia)

- NICE recommends the same psychological interventions as for MDD.
- CBASP (Cognitive Behavioural Analysis System of Psychotherapy, McCullough 2000) was developed specifically for chronic depression. Combines behavioural, cognitive, and interpersonal elements. The Keller et al. (2000) NEJM trial showed CBASP + nefazodone > either alone.
- Schema Therapy may be indicated when early maladaptive schemas maintain chronic depressive patterns. Young identifies Defectiveness/Shame, Emotional Deprivation, and Failure schemas as common in chronic depression.

Treatment-Resistant Depression

- NICE defines treatment resistance as failure to respond to two adequate antidepressant trials.
- Augmentation with psychological therapy is recommended: CBT is the best studied adjunct (Wiles et al., 2013: CoBalT trial — CBT added to medication improved outcomes in treatment-resistant depression, NNT = 4).
- STPP and schema therapy are options when characterological factors are identified.

> EXAM STRATEGY
>
> For treatment-resistant depression, always mention the CoBalT trial (Wiles et al., 2013). This is the highest-quality evidence for adding CBT to antidepressants in treatment-resistant cases.

Depression Therapy Comparison

Therapy	Effect Size (d) vs. Control	Key Trial	Strengths	Limitations
CBT	0.71	Rush et al. 1977; Elkin et al. 1989	Most studied; structured; skills-based	May be less effective in severe depression (NIMH TDCRP)
IPT	0.67	Elkin et al. 1989	Addresses interpersonal context; time-limited	Limited therapist availability; narrow focus
BA	0.74	Dimidjian et al. 2006	Simpler to train; effective in severe depression	Less evidence for relapse prevention
STPP	0.69	Leichsenring et al. 2004	Addresses personality factors; long-term gains	Longer training; fewer RCTs
MBCT	Relapse RR 0.69	Kuyken et al. 2016	Relapse prevention; group format	Not for acute depression; requires meditation practice
CBASP	—	Keller et al. 2000 (NEJM)	Designed for chronic depression	Complex; requires specific training

3. ANXIETY DISORDERS

3.1 Generalised Anxiety Disorder (GAD)

First-line: CBT (Borkovec model — targets intolerance of uncertainty, worry as avoidance, positive beliefs about worry). NICE CG113 recommends individual CBT (16–20 sessions) or applied relaxation.

Applied Relaxation (Ost, 1987): Progressive muscle relaxation → cue-controlled relaxation → differential relaxation → rapid relaxation → application in anxiety-provoking situations. Equivalent to CBT in Ost's own trials; NICE lists it as an alternative first-line.

Second-line: Short-term psychodynamic psychotherapy, mindfulness-based interventions.

> EXAM PEARL
>
> The key GAD cognitive model is the Dugas et al. (1998) model centring on intolerance of uncertainty. The Wells (2004) metacognitive model targets positive meta-beliefs about worry ("worrying helps me cope") and negative meta-beliefs ("worry is uncontrollable"). Both are used in CBT for GAD.

3.2 Social Anxiety Disorder

First-line: Individual CBT based on the Clark and Wells (1995) model. This model identifies three maintaining factors: self-focused attention, safety behaviours, and post-event rumination.

- Clark et al. (2006): Individual CBT superior to fluoxetine, group CBT, and placebo. One of the largest effect sizes in the anxiety therapy literature (d = 1.20 vs. waiting list).
- Stangier et al. (2003): Individual CBT superior to group CBT for social anxiety.
- NICE CG159: Individual CBT (Clark and Wells model) recommended as first-line. Group CBT is an alternative.

Second-line: Short-term psychodynamic psychotherapy. Leichsenring et al. (2013, JAMA): STPP non-inferior to CBT at 6–month follow-up.

> CLINICAL ANCHOR
>
> The Clark and Wells model is the single most examined CBT model for social anxiety. In a viva, you must be able to describe the three maintaining factors: (1) self-focused attention shifts processing inward, creating distorted self-images; (2) safety behaviours prevent disconfirmation of feared outcomes; (3) anticipatory and post-event processing maintain negative beliefs.

3.3 Specific Phobias

First-line: Exposure therapy (in vivo graded exposure). The most parsimonious and effective treatment in all of psychotherapy — single-session exposure (Ost, 1989) can produce clinically significant improvement in 1–3 hours.

- Mechanism: habituation (fear response diminishes with sustained exposure) and inhibitory learning (new non-threat association formed alongside the fear memory).
- Systematic desensitisation (Wolpe, 1958) is the historical precursor — combines relaxation with graded imaginal exposure. Largely replaced by in vivo exposure.

> EXAM PEARL
>
> Applied tension (Ost & Sterner, 1987) is the treatment of choice for blood-injection-injury phobia specifically. Unlike other phobias (which involve sympathetic activation), BII phobia involves a vasovagal (parasympathetic) response. Applied tension counteracts this by increasing blood pressure during exposure.

3.4 Panic Disorder

First-line: CBT based on the Clark (1986) cognitive model. The model: bodily sensations → catastrophic misinterpretation ("I'm having a heart attack") → increased anxiety → increased bodily sensations → positive feedback loop.

- Clark et al. (1994, 1999): CBT produced 80–90% panic-free rates at post-treatment. Superior to imipramine, applied relaxation, and waitlist.
- Key techniques: cognitive restructuring of catastrophic misinterpretations, interoceptive exposure (deliberately inducing feared sensations — hyperventilation, spinning, caffeine), behavioural experiments to test predictions, dropping safety behaviours.
- NICE CG113: CBT is first-line. 7–14 sessions.

> EXAM PEARL
>
> Interoceptive exposure is the technique that distinguishes panic disorder CBT from standard anxiety CBT. The patient deliberately hyperventilates, breathes through a straw, spins in a chair, or runs up stairs to trigger feared sensations in a controlled setting — this breaks the catastrophic misinterpretation cycle.

3.5 Agoraphobia

First-line: CBT with graded in vivo exposure to avoided situations. Often treated alongside panic disorder (as per Clark model). The behavioural component (systematic exposure to public transport, crowds, open spaces, being alone outside) is essential.

Anxiety Disorders x Therapy Matrix

Disorder	1st Line Therapy	Key Model/Protocol	2nd Line	NNT (approx.)
GAD	CBT, Applied Relaxation	Borkovec; Dugas IU model; Wells metacognitive	STPP, mindfulness	5–6
Social Anxiety	Individual CBT (Clark & Wells)	Self-focused attention, safety behaviours, post-event rumination	STPP (Leichsenring 2013)	3–4
Specific Phobia	In vivo exposure	Habituation / inhibitory learning	Systematic desensitisation	2–3
Panic Disorder	CBT (Clark model)	Catastrophic misinterpretation + interoceptive exposure	Applied relaxation	3–4
Agoraphobia	CBT + graded exposure	Behavioural avoidance model	Guided self-help exposure	4–5

4. OCD AND RELATED DISORDERS

4.1 Obsessive-Compulsive Disorder (OCD)

First-line: CBT with Exposure and Response Prevention (ERP). The single most effective psychological treatment for OCD for over four decades.

- Salkovskis (1985, 1999) cognitive model: intrusive thoughts are universal (90% of the general population); what differentiates OCD is the appraisal of

personal responsibility for preventing harm. This appraisal triggers neutralising behaviour (compulsions) which provides temporary relief but maintains the cycle.

- ERP protocol: hierarchical exposure to obsessional triggers + absolute prevention of compulsive rituals. The patient learns that (a) anxiety naturally habituates and (b) the feared catastrophe does not occur.
- NICE CG31: ERP with at least 10 hours of therapist contact. For mild-moderate OCD: low-intensity CBT including ERP. For moderate-severe: intensive CBT including ERP OR SSRI.
- Foa et al. (2005): ERP alone = clomipramine alone; ERP + clomipramine was not significantly better than ERP alone. This is one of the few disorders where psychotherapy alone matches pharmacotherapy for moderate-severe illness.

> EXAM PEARL
>
> The Salkovskis model distinguishes between intrusions (universal) and obsessions (pathological). The key pathological appraisal is inflated responsibility — "If I don't check the stove, the house will burn down and it will be MY fault." Treatment targets this appraisal via behavioural experiments and cognitive restructuring alongside ERP.

Second-line: SSRIs (at higher doses than for depression). Third-line: clomipramine. Augmentation with antipsychotics (aripiprazole) or intensive residential ERP for treatment-resistant cases.

> CLINICAL ANCHOR
>
> ERP requires specialist training. The most common error by generalist therapists is conducting "exposure without response prevention" — showing the patient anxiety-provoking stimuli but allowing partial ritualising. This produces sensitisation, not habituation. Full response prevention is essential.

4.2 Body Dysmorphic Disorder (BDD)

First-line: CBT adapted for BDD (Veale & Neziroglu, 2010). Targets: excessive self-focused attention, checking/reassurance-seeking, mirror avoidance or mirror gazing, and appearance-related beliefs. Includes ERP for checking behaviours.

Second-line: SSRIs (higher doses).

4.3 Hoarding Disorder

First-line: CBT adapted for hoarding (Steketee & Frost, 2007). Unique features: motivational interviewing (insight is often poor), cognitive restructuring targeting beliefs about possessions ("I might need this someday"), exposure to discarding, and organisational skills training.

- Response rates are lower than for OCD proper (~30% achieve clinically significant improvement vs. ~60% for OCD with ERP).

---

5. TRAUMA AND STRESSOR-RELATED DISORDERS

5.1 Post-Traumatic Stress Disorder (PTSD)

NICE NG116 Recommendations

First-line: Trauma-focused psychological therapy:

1. **Trauma-focused CBT (TF-CBT)** — includes prolonged exposure (PE) and cognitive processing therapy (CPT)
2. **Eye Movement Desensitisation and Reprocessing (EMDR)**

Both should be offered as first-line treatments. Medication (SSRI) is second-line (recommended only when the patient declines or does not respond to psychological therapy).

Prolonged Exposure (PE, Foa et al., 2007)

- The most extensively studied PTSD psychotherapy. Mechanism: emotional processing theory — the fear structure is activated through imaginal re-experiencing, and corrective information is incorporated.
- Protocol: 8–15 sessions of 90 minutes. Imaginal exposure (repeated detailed narrative of the trauma) + in vivo exposure to avoided trauma-related cues.
- Powers et al. (2010) meta-analysis: d = 1.08 vs. waitlist.

Cognitive Processing Therapy (CPT, Resick et al., 2002)

- Focuses on maladaptive cognitions ("stuck points") about the trauma — specifically beliefs about safety, trust, power/control, esteem, and intimacy.
- 12 sessions, structured. Can be delivered without a written trauma account.
- Resick et al. (2012): CPT equivalent to PE; both superior to waitlist.

EMDR (Shapiro, 1989)

- Bilateral stimulation (eye movements, taps, or tones) while the patient holds a traumatic memory in mind. Proposed mechanism: working memory taxation reduces the vividness and emotionality of the traumatic memory.
- NICE recommends 8–12 sessions. Evidence quality is comparable to TF-CBT (Bisson et al., 2007 Cochrane review).

> EXAM PEARL
>
> The mechanism of EMDR remains debated. The eye-movement component may work through working memory taxation (van den Hout & Engelhard, 2012) — holding the traumatic image while performing a concurrent task degrades the sensory quality of the memory. Some argue the eye movements are incidental and the exposure/processing component does the work. Examiners want you to acknowledge this debate.

5.2 Complex PTSD

Phase-based treatment is recommended (Cloitre et al., 2011 ISTSS guidelines):

- **Phase 1:** Stabilisation and skills (emotion regulation, grounding, safety)
- **Phase 2:** Trauma processing (using PE, CPT, EMDR, or imagery rescripting)
- **Phase 3:** Reconnection and rehabilitation (relationships, identity, meaning)

STAIR (Skills Training in Affective and Interpersonal Regulation) + Prolonged Exposure is the best-studied phase-based protocol (Cloitre et al., 2010).

> CLINICAL ANCHOR
>
> Schema Therapy is increasingly used for complex PTSD, particularly when early relational trauma has produced pervasive schemas (Abandonment, Mistrust/Abuse, Defectiveness). Young emphasises that trauma processing via imagery rescripting in ST is conducted only after schema assessment and stabilisation — never in early sessions.

5.3 Adjustment Disorder

- NICE does not have specific guidelines; general principles of supportive counselling, brief CBT, or brief psychodynamic therapy apply.
- Problem-solving therapy has the strongest evidence base (Cuijpers et al., 2020).

6. PERSONALITY DISORDERS

6.1 Borderline Personality Disorder (BPD)

The most extensively studied personality disorder in psychotherapy research. Four evidence-based treatments exist, each with distinct theoretical models.

BPD Treatments Head-to-Head Comparison

Feature	DBT (Linehan)	MBT (Bateman & Fonagy)	TFP (Kernberg)	ST (Young)
Theoretical model	Biosocial: biological vulnerability + invalidating environment	Attachment: impaired mentalisation from insecure attachment	Object relations: identity diffusion, primitive defences, low-level personality organisation	Schema: EMSs from unmet childhood needs; 5 modes in BPD
Primary target	Emotion dysregulation, suicidal/self-harm behaviour	Mentalisation (capacity to understand behaviour in terms of mental states)	Identity integration, modulation of affect, object constancy	Schema healing, mode change, limited reparenting
Therapist stance	Radical acceptance + push for change; irreverent + reciprocal	Not-knowing, curious, affect-focused	Neutral, interpretive, maintains technical neutrality	Limited reparenting, empathic confrontation
Structure	Individual therapy + group skills + phone coaching + consultation team	Individual (1×/week) + group analytic therapy (1×/week)	Individual (2×/week)	Individual (1–2×/week), can include group
Key techniques	Chain analysis, diary cards, skills training (4 modules), validation, DBT strategies	Clarification, elaboration, challenge; mentalising the transference	Transference interpretation, clarification, confrontation	Imagery rescripting, chair work, flash cards, pattern-breaking, mode dialogues

Feature	DBT (Linehan)	MBT (Bateman & Fonagy)	TFP (Kernberg)	ST (Young)
Duration	1 year standard programme	18 months intensive; 18 months stepdown	1–3 years	2–3 years
Key trial	Linehan et al. (1991, 2006)	Bateman & Fonagy (1999, 2001, 2008)	Doering et al. (2010); Clarkin et al. (2007)	Giesen-Bley et al. (2006); Farrell et al. (2009); Nadort et al. (2009)
Primary outcome evidence	Reduced self-harm, hospitalisations, suicidality	Reduced self-harm, hospitalisations, depression, improved social function	Improved personality organisation, reduced aggression	Greater schema change, higher recovery rates (Giesen-Bley: 45% ST vs. 24% TFP recovery)
NICE recommendation	Recommended	Recommended	—	—
Ideal patient	High-risk, suicidal, emotionally dysregulated	Comorbid depression, interpersonal chaos, low mentalisation	Higher-functioning BPD with identity confusion, aggression	Chronic BPD with prominent schemas; "treatment failures" from CBT

EXAM PEARL

The Giesen-Bley et al. (2006) RCT compared ST vs. TFP for BPD. Schema therapy showed higher recovery rates (45% vs. 24%), lower dropout (27% vs. 50%), and greater improvement across all BPD symptoms. This is the only head-to-head RCT between two specialist BPD treatments showing superiority of one over the other.

EXAM PEARL

DBT has four modes (individual therapy, group skills training, phone coaching, therapist consultation team). Each mode addresses a different function. The most commonly tested error in exams is confusing DBT group skills training with group therapy — DBT skills groups are psychoeducational, not process-oriented.

CLINICAL ANCHOR

The DBT target hierarchy is one of the most examined concepts in BPD therapy: (1) life-threatening behaviours, (2) therapy-interfering behaviours, (3) quality-of-life-interfering behaviours, (4) increasing behavioural skills. This hierarchy governs every individual therapy session — the therapist always addresses higher-priority targets first.

NICE CG78 for BPD

- Do NOT use brief psychological interventions (<3 months) for BPD
- Do NOT use medication as primary treatment
- Offer comprehensive treatment programmes (DBT or MBT recommended)
- Duration: at least 1 year for moderate-severe BPD
- Crisis planning and management are essential components

6.2 Antisocial Personality Disorder (ASPD)

- NICE CG77: No specific psychotherapy has strong evidence. CBT-based programmes in forensic settings have limited evidence.
- Beck's CT-PD approach: risk-benefit analysis of antisocial behaviour, "choice review" exercises, building cognitive hierarchy of moral development.
- Key finding: depressed ASPD patients may respond to treatment; non-depressed ASPD patients generally do not (Woody et al., 1985).

EXAM STRATEGY

When asked about ASPD treatment, acknowledge the limited evidence base. Mention that Woody et al. (1985) found that comorbid depression predicts treatment response, and that some forensic CBT programmes show modest effects on reoffending.

6.3 Narcissistic Personality Disorder (NPD)

- No NICE guideline. Limited RCT evidence.
- Schema Therapy (Young): treats the Lonely Child mode (underlying Emotional Deprivation, Defectiveness), confronts the Self-Aggrandizer mode, builds Healthy Adult. Typically 40+ sessions.
- CT-PD (Beck): avoids direct challenge to grandiosity early; uses cost-benefit analysis; builds alternative sources of self-esteem.
- Psychodynamic: Kohut (self-psychology — empathic immersion, selfobject transference) vs. Kernberg (TFP — confrontation of narcissistic defences). Fundamental theoretical disagreement about technique.

EXAM PEARL

The Kohut vs. Kernberg debate is a classic exam topic. Kohut advocates empathy and does NOT confront grandiosity (seeing it as a developmental arrest requiring selfobject experiences). Kernberg advocates confrontation of narcissistic defences (seeing grandiosity as a pathological defence against primitive rage and envy). Schema Therapy takes a middle position: empathise with the Lonely Child while setting limits on the Self-Aggrandizer.

6.4 Avoidant Personality Disorder (AvPD)

- CBT: graduated exposure, positive data logs, schema change methods for core beliefs ("I am socially inept"). Must address cognitive, behavioural, AND emotional avoidance (Beck CT-PD notes three types of avoidance).
- Schema Therapy: targets Defectiveness/Shame, Social Isolation schemas; emphasises exposure to intimate contact and building tolerance for negative

affect.

- NICE does not have a specific AvPD guideline; managed under personality disorder pathways.

6.5 Obsessive-Compulsive Personality Disorder (OCPD)

- CT-PD (Beck): businesslike therapeutic relationship; behavioural experiments comparing perfectionistic vs. "good enough" strategies; advantage-disadvantage analysis of rigid rules.
- Schema Therapy: targets Unrelenting Standards and Punitiveness schemas.
- Psychodynamic: addresses underlying helplessness/defectiveness compensated by control and orderliness.

6.6 Other Personality Disorders

PD	Primary Approach	Key Points
Dependent PD	CBT (graded autonomy tasks, assertiveness training), Schema Therapy (Dependence schema)	Treatment paradox: increasing competence triggers abandonment fear
Histrionic PD	CBT (reinforce competence, address global thinking style), psychodynamic	Use dramatic methods; avoid saviour role
Paranoid PD	CBT (continuum technique for trust, assertiveness training), psychodynamic	Build trust slowly; increase self-efficacy first
Schizoid PD	CBT (advantages/disadvantages of isolation), psychodynamic	Engagement itself is the primary challenge
Schizotypal PD	CBT (normalisation with prevalence data, dropping safety behaviours)	Distinguish from psychosis; comorbid depression predicts response

7. EATING DISORDERS

7.1 Anorexia Nervosa (AN)

First-line (adults): No psychotherapy has robust evidence of superiority. NICE NG69 recommends:

- Individual eating disorder-focused CBT (CBT-ED)
- MANTRA (Maudsley Anorexia Nervosa Treatment for Adults — Schmidt & Treasure, 2006)
- SSCM (Specialist Supportive Clinical Management)

First-line (children/adolescents): Family-Based Treatment (FBT, Maudsley model — Lock & Le Grange, 2013). Three phases: (1) weight restoration with parents in charge of feeding, (2) gradual return of eating control to adolescent, (3) identity and developmental issues. The Eisler et al. (2007) trial and Lock et al. (2010) 4-year follow-up support FBT.

> EXAM PEARL
>
> FBT (the Maudsley approach) for adolescent AN is the only eating disorder psychotherapy with strong evidence for a specific age group. It agnosticises about aetiology and places parents as the primary agents of change. This is a major shift from earlier psychodynamic approaches that implicated parents in causation.

Second-line (adults): Focal psychodynamic psychotherapy, Schema Therapy (targeting Defectiveness/Shame, Subjugation).

7.2 Bulimia Nervosa (BN)

First-line: CBT-E (Enhanced CBT, Fairburn et al., 2003, 2009). The transdiagnostic CBT model treats all eating disorders through four maintaining mechanisms: clinical perfectionism, core low self-esteem, mood intolerance, and interpersonal difficulties.

- 20 sessions over 20 weeks (focused form) or extended form (addressing the four maintaining mechanisms).
- Fairburn et al. (2009): CBT-E produced remission in approximately 50% of BN patients.
- NICE NG69: CBT-E is first-line. If no response after 4 sessions, consider IPT.

Second-line: IPT (Agras et al., 2000 — IPT equivalent to CBT at 12–month follow-up, though slower in onset).

7.3 Binge Eating Disorder (BED)

First-line: CBT (individual or group), guided self-help CBT. NICE NG69.

Second-line: IPT. Wilfley et al. (2002): Group CBT = Group IPT at post-treatment and 1–year follow-up.

Eating Disorder Therapy Comparison

Disorder	1st Line	2nd Line	Key Trial	Duration
AN (adolescent)	FBT (Maudsley)	Individual therapy	Lock et al. 2010	6–12 months
AN (adult)	CBT-ED, MANTRA, SSCM	Focal psychodynamic, ST	Zipfel et al. 2014 (ANTOP trial)	40+ sessions
BN	CBT-E (Fairburn)	IPT	Fairburn et al. 2009	20 sessions/20 weeks
BED	CBT, guided self-help	IPT	Wilfley et al. 2002	16–20 sessions

> CLINICAL ANCHOR
>
> CBT-E (the "E" stands for Enhanced) is transdiagnostic — it can be applied to AN, BN, and BED using the same framework. The focused form targets the core eating disorder psychopathology. The broad form also addresses clinical perfectionism, core low self-esteem, mood intolerance, and interpersonal difficulties when these maintain the eating disorder.

8. SUBSTANCE USE DISORDERS

8.1 First-Line Approaches

Motivational Interviewing (MI, Miller & Rollnick, 1991, 2013)

- Not a therapy per se but a communication style. Four processes: engaging, focusing, evoking, planning. Core spirit: partnership, acceptance, compassion, evocation.
- NICE recommends MI as a brief intervention for all substance use disorders, particularly in early stages of change.
- Works through resolving ambivalence — the therapist selectively elicits and reinforces "change talk" (DARN-CAT: Desire, Ability, Reasons, Need, Commitment, Activation, Taking steps).

CBT Relapse Prevention (Marlatt & Gordon, 1985; Marlatt & Donovan, 2005)

- Identifies high-risk situations, coping skills deficits, and the Abstinence Violation Effect (AVE — a single lapse is catastrophised into full relapse).
- Key techniques: functional analysis of substance use, coping skills training, apparently irrelevant decisions analysis, urge surfing (mindfulness-based).
- Irvin et al. (1999) meta-analysis: CBT relapse prevention has the most consistent evidence base across substances.

> EXAM PEARL
>
> The Abstinence Violation Effect (AVE) is a critical concept: after a lapse, the patient attributes it to internal, stable, global causes ("I'm an addict, I'll never change") rather than external, unstable, specific causes ("I was in a high-risk situation without coping skills"). CBT relapse prevention specifically targets this cognitive distortion.

8.2 Other Evidence-Based Approaches

Community Reinforcement Approach (CRA, Hunt & Azrin, 1973)

- Rearranges environmental contingencies so that sobriety is more rewarding than substance use. Includes relationship counselling, job skills, social/recreational counselling.
- NICE recommends for alcohol dependence.

Contingency Management (CM)

- Provides tangible reinforcers (vouchers, prizes) for verified abstinence (negative urine drug screens).
- NICE recommends for opioid and stimulant use disorders. Strong evidence for cocaine and methamphetamine use (where no effective pharmacotherapy exists).

Twelve-Step Facilitation (TSF)

- Structured therapy to encourage engagement with Alcoholics Anonymous or other 12–step programmes.
- Project MATCH (1997): TSF equivalent to CBT and MET for alcohol dependence at 1–year and 3–year follow-up. TSF may have advantages for patients with high-severity dependence and low social support.

EXAM STRATEGY

For substance use disorders, the classic viva trap is asking whether AA/12–step programmes "work." The evidence-based answer: Project MATCH (1997) showed TSF was equivalent to CBT and MET. Cochrane reviews show moderate evidence supporting TSF. The mechanism is likely increased social support and abstinence-reinforcing norms.

9. PSYCHOTIC DISORDERS

9.1 CBT for Psychosis (CBTp)

- NICE CG178: CBTp should be offered to all people with schizophrenia. At least 16 planned sessions.
- Mechanism: helps patients develop alternative explanations for psychotic experiences, reduces distress associated with delusions/hallucinations, and addresses secondary problems (depression, self-esteem).
- Morrison et al. (2014): CBTp reduces symptom severity with small-to-moderate effect size (d = 0.33–0.44 vs. treatment as usual).
- CBTp does NOT aim to eliminate hallucinations or delusions. The target is distress and functional impairment.

EXAM PEARL

The effect size for CBTp (d = 0.33–0.44) is modest compared to CBT for depression or anxiety. However, given that this is an adjunct to antipsychotics for a chronic illness, this is clinically meaningful. Jauhar et al. (2014) meta-analysis found smaller effects than earlier reviews, generating debate. The NICE recommendation stands — CBTp should still be offered.

9.2 Family Intervention

- NICE CG178: Family intervention should be offered to all families of people with schizophrenia (at least 10 sessions over 3+ months).

- Based on Expressed Emotion (EE) research (Brown et al., 1972; Vaughn & Leff, 1976): high EE (criticism, hostility, emotional over-involvement) predicts relapse.
- Pilling et al. (2002) meta-analysis: family intervention reduced relapse rates by approximately 50%.
- Components: psychoeducation, communication skills, problem-solving, crisis management, reducing high EE.

9.3 Social Skills Training

- Moderately effective for improving social functioning (Kurtz & Mueser, 2008 meta-analysis: d = 0.52 for social functioning).
- Not specifically recommended by NICE as a standalone intervention but forms part of comprehensive psychosocial rehabilitation.

> CLINICAL ANCHOR
>
> Recovery-Oriented Cognitive Therapy (CT-R, Beck et al.) is an emerging approach that focuses on adaptive beliefs, aspirations, and strengths rather than targeting psychotic symptoms directly. It represents a shift in CBTp from deficit-focused to strengths-based work.

10. BIPOLAR DISORDER

NICE CG185: Psychological interventions are recommended as adjuncts to mood stabilisers, not as standalone treatments.

10.1 Psychoeducation

- The simplest and most consistently effective psychological intervention for bipolar disorder.
- Colom et al. (2003, 2009): Group psychoeducation (21 sessions) significantly reduced relapse rates at 2–year and 5–year follow-up.

- Content: illness understanding, medication adherence, early warning sign identification, lifestyle regulation, relapse prevention planning.

10.2 Interpersonal and Social Rhythm Therapy (IPSRT, Frank et al., 2005)

- Integrates IPT with social rhythm regulation. Based on the social zeitgeber theory: disruptions to social rhythms (sleep-wake cycles, mealtimes, activity patterns) destabilise circadian rhythms, triggering mood episodes.
- Social Rhythm Metric tracks daily routines. The therapy stabilises rhythms while addressing interpersonal problems.
- Frank et al. (2005): IPSRT during acute treatment led to longer time to recurrence in the maintenance phase.

> EXAM PEARL
>
> IPSRT is the only psychotherapy developed specifically for bipolar disorder from the ground up (not adapted from a depression therapy). The social zeitgeber hypothesis that underpins it — that life events destabilise social rhythms, which destabilise circadian rhythms, which trigger mood episodes — is frequently examined.

10.3 CBT for Bipolar Disorder

- Lam et al. (2003): CBT reduced relapse rates at 12 months. However, Scott et al. (2006) found CBT was only effective for patients with fewer than 12 prior episodes.
- NICE recommends CBT (at least 16 sessions) focusing on relapse prevention, medication adherence, and managing residual symptoms.

10.4 Family-Focused Therapy (FFT, Miklowitz et al., 2003)

- 21 sessions over 9 months: psychoeducation, communication enhancement training, problem-solving skills.

- Miklowitz et al. (2003): FFT + pharmacotherapy reduced relapse rates compared to crisis management + pharmacotherapy.
- Particularly effective for younger patients and those in high-EE families.

> CLINICAL ANCHOR
>
> For bipolar disorder, the evidence hierarchy for psychological interventions is: (1) psychoeducation (strongest, simplest), (2) IPSRT, (3) CBT, (4) FFT. All are adjunctive to pharmacotherapy. No psychological intervention replaces mood stabilisers.

11. CHILD AND ADOLESCENT DISORDERS

11.1 ADHD

First-line: Parent training / behavioural management programmes (NICE NG87).

- For preschool children: parent training is first-line before any medication consideration.
- For school-age children: parent training + school-based behavioural interventions as first-line for mild-moderate; medication (methylphenidate) for moderate-severe.
- Evidence: MTA Study (1999) — medication > behavioural treatment > community care for core ADHD symptoms; combined treatment = medication for core symptoms but superior for associated problems (oppositional behaviour, parent-child relationship).

> EXAM PEARL
>
> The MTA Study (Multimodal Treatment of ADHD, 1999) is the landmark trial. At 14 months: medication management (carefully titrated) > intensive behavioural treatment for core ADHD symptoms. However, at 3-year follow-up (Jensen et al., 2007), the medication advantage had diminished. This is frequently examined — always cite both time points.

11.2 Conduct Disorder

First-line (children): Parent training programmes (e.g., Triple P, Incredible Years, Parent Management Training — Oregon Model).

First-line (adolescents):

- Multisystemic Therapy (MST, Henggeler): intensive family and community-based treatment addressing multiple systems (family, school, peer, community). Evidence: Henggeler et al. (1998) reduced reoffending.
- Parent-Child Interaction Therapy (PCIT, Eyberg): live-coached parent-child interactions targeting disruptive behaviour.

11.3 Separation Anxiety Disorder

First-line: CBT adapted for children (e.g., Coping Cat programme, Kendall 1994). Includes psychoeducation, coping skills, graded exposure to separation situations. Kendall et al. (1997): 71% of treated children no longer met diagnostic criteria at post-treatment.

11.4 Adolescent Depression

First-line: CBT (adapted for adolescents), IPT-A (interpersonal therapy for adolescents — Mufson et al., 2004).

- TADS Study (2004): CBT + fluoxetine > fluoxetine alone > CBT alone > placebo. Combined treatment is the gold standard for moderate-severe adolescent depression.

> EXAM PEARL
>
> The TADS study (Treatment of Adolescents with Depression Study, 2004) is essential for adolescent depression. Combined CBT + fluoxetine had a response rate of 71% vs. fluoxetine alone 61% vs. CBT alone 43% vs. placebo 35%. The surprisingly low CBT-alone response rate was controversial and differs from adult findings.

Child/Adolescent Therapy by Disorder

Disorder	1st Line	2nd Line	Key Trial
ADHD (preschool)	Parent training	—	NICE NG87
ADHD (school-age)	Parent training + school BT; medication for moderate-severe	CBT skills training	MTA Study 1999
Conduct Disorder (child)	Parent training (Triple P, Incredible Years)	—	Webster-Stratton 1998
Conduct Disorder (adolescent)	MST, PCIT	FFT (Functional Family Therapy)	Henggeler et al. 1998
Separation Anxiety	CBT (Coping Cat)	Family CBT	Kendall et al. 1997
Adolescent Depression	CBT + SSRI (combined), IPT-A	CBT alone, fluoxetine alone	TADS 2004
Adolescent BPD features	DBT-A (Rathus & Miller)	MBT-A	Mehlum et al. 2014
Adolescent Self-Harm	DBT-A	MBT-A, CBT	Mehlum et al. 2014
Adolescent Anxiety	CBT (Coping Cat / FRIENDS)	Family CBT	Kendall et al. 1997
Adolescent AN	FBT (Maudsley)	Individual therapy	Lock et al. 2010

CLINICAL ANCHOR

DBT for adolescents (Rathus & Miller) differs from adult DBT in several key ways: a fifth skills module (Walking the Middle Path) addresses parent-teen dialectical dilemmas, parents attend skills groups alongside adolescents, and the programme duration is 24 weeks rather than 1 year.

12. COUPLES AND RELATIONAL PROBLEMS

12.1 Gottman Method Couples Therapy

- Based on Gottman's research on marital stability (Sound Relationship House theory).
- Key concepts: "Four Horsemen" (criticism, contempt, defensiveness, stonewalling), 5:1 positive-to-negative ratio, turning toward vs. turning away from bids for connection.
- Assessment-based: extensive evaluation (questionnaires, oral history interview, conflict discussion) before intervention.

12.2 Emotionally Focused Therapy for Couples (EFT-C, Johnson, 2004)

- Based on attachment theory (Bowlby). Distress occurs when attachment bonds are threatened; couples get stuck in negative interaction cycles (pursue-withdraw being the most common).
- Three stages: (1) de-escalation of negative cycles, (2) restructuring attachment bonds (accessing and expressing primary emotions), (3) consolidation.
- Johnson et al. (1999) meta-analysis: d = 1.31 (large effect size). 70–75% of couples move from distress to recovery.

12.3 Behavioural Couples Therapy (BCT, Jacobson & Christensen)

- Based on social exchange theory and operant conditioning. Targets behavioural exchanges between partners.
- Techniques: behavioural exchange (caring days), communication training, problem-solving training.

- Integrative BCT (IBCT, Christensen et al., 2004) adds acceptance strategies to traditional BCT. Christensen et al. (2004): IBCT > traditional BCT at 2–year follow-up.

> EXAM STRATEGY
>
> When asked about couples therapy for depression, note that Behavioural Couples Therapy has evidence for depression where relationship distress is a maintaining factor (Barbato & D'Avanzo, 2008, Cochrane review). EFT-C has the largest effect sizes in the couples therapy literature but fewer independent replications.

MASTER TABLE: DISORDER-SPECIFIC

THERAPY MAP

#	Disorder	1st Line Therapy	2nd Line	Emerging / Adjunct	NICE Guideline	Key Trial
1	MDD (mild-moderate)	CBT, BA, IPT	Guided self-help, STPP	MBCT (relapse prevention)	CG90/NG222	Rush 1977; Elkin 1989
2	MDD (severe)	CBT + SSRI, IPT + SSRI	BA + SSRI	CBASP	CG90/NG222	Cuijpers 2020
3	Persistent Depressive Disorder	CBT, IPT	CBASP, Schema Therapy	MBCT	(as MDD)	Keller 2000
4	Treatment-Resistant Depression	CBT augmentation	STPP, Schema Therapy	Ketamine-assisted therapy	CG90	Wiles 2013 (CoBalT)
5	GAD	CBT, Applied Relaxation	STPP, mindfulness	Metacognitive Therapy (Wells)	CG113	Borkovec 1995
6	Social Anxiety Disorder	CBT (Clark & Wells)	STPP	Internet-delivered CBT	CG159	Clark 2006
7	Specific Phobia	In vivo exposure	Systematic desensitisation	Virtual reality exposure	—	Ost 1989
8	Panic Disorder	CBT (Clark model)	Applied Relaxation	Internet-delivered CBT	CG113	Clark 1994

#	Disorder	1st Line Therapy	2nd Line	Emerging / Adjunct	NICE Guideline	Key Trial
9	Agoraphobia	CBT + graded exposure	Guided self-help exposure	Virtual reality exposure	CG113	—
10	OCD	CBT with ERP	SSRI (high dose)	Intensive residential ERP	CG31	Foa 2005
11	BDD	CBT (adapted)	SSRI	—	CG31	Veale 2010
12	Hoarding Disorder	CBT (adapted) + MI	SSRI	—	—	Steketee 2007
13	PTSD	TF-CBT (PE, CPT), EMDR	SSRI (sertraline, paroxetine)	Imagery rescripting, MDMA-AT	NG116	Powers 2010; Bisson 2007
14	Complex PTSD	Phase-based (STAIR+PE)	Schema Therapy	EMDR + stabilisation	(NG116)	Cloitre 2010
15	Adjustment Disorder	Supportive counselling, brief CBT	Problem-solving therapy	—	—	Cuijpers 2020
16	BPD	DBT, MBT	TFP, Schema Therapy	Group Schema Therapy	CG78	Linehan 2006; Bateman 2008; Giesen-Bley 2006

#	Disorder	1st Line Therapy	2nd Line	Emerging / Adjunct	NICE Guideline	Key Trial
17	ASPD	Forensic CBT programmes	MI + anger management	—	CG77	Woody 1985
18	NPD	Schema Therapy	Psychodynamic (Kohut/Kernberg)	CT-PD (Beck)	—	—
19	AvPD	CBT, Schema Therapy	STPP	Group CBT	—	—
20	AN (adolescent)	FBT (Maudsley)	Individual therapy	—	NG69	Lock 2010
21	AN (adult)	CBT-ED, MANTRA, SSCM	Focal psychodynamic	Schema Therapy	NG69	Zipfel 2014 (ANTOP)
22	BN	CBT-E (Fairburn)	IPT	Guided self-help	NG69	Fairburn 2009
23	BED	CBT, guided self-help	IPT	—	NG69	Wilfley 2002
24	Alcohol Use Disorder	MI, CBT relapse prevention	CRA, TSF	Contingency management	CG115	Project MATCH 1997
25	Opioid Use Disorder	CBT relapse prevention + OAT	Contingency management	—	CG51	—

#	Disorder	1st Line Therapy	2nd Line	Emerging / Adjunct	NICE Guideline	Key Trial
26	Stimulant Use Disorder	CBT, Contingency management	CRA	MI	—	—
27	Schizophrenia	CBTp + family intervention	Social skills training	CT-R (Beck)	CG178	Morrison 2014; Pilling 2002
28	Bipolar Disorder	Psychoeducation + mood stabiliser	IPSRT, CBT, FFT	—	CG185	Colom 2003; Frank 2005
29	ADHD (child)	Parent training, school BT	CBT skills (adjunct)	—	NG87	MTA Study 1999
30	Conduct Disorder	Parent training, MST	PCIT, FFT	—	CG158	Henggeler 1998
31	Adolescent Depression	CBT + SSRI, IPT-A	CBT alone	DBT-A (with self-harm)	NG134	TADS 2004
32	Couples Distress	EFT-C, Gottman Method	BCT, IBCT	—	—	Johnson 1999

Viva Practice Questions

1. **Compare CBT and IPT for depression. When would you choose one over the other?**
2. **Describe the Clark and Wells model for social anxiety and how it informs treatment.**

3. What is the evidence base for ERP in OCD? Is it superior to pharmacotherapy?
4. Compare the four evidence-based treatments for BPD (DBT, MBT, TFP, ST). Which has the strongest evidence?
5. What is the role of psychological therapy in schizophrenia? Describe CBTp.
6. Explain IPSRT for bipolar disorder. What theoretical model underpins it?
7. A patient with PTSD is referred for psychological therapy. Walk through the NICE-recommended options.
8. What is MBCT and for whom is it indicated? When would you NOT recommend it?
9. Describe the Maudsley model (FBT) for adolescent anorexia nervosa.
10. What psychological interventions are effective for substance use disorders?
11. How does CBT for personality disorders differ from CBT for Axis I disorders? (Beck CT-PD framework)
12. A patient with BPD is in crisis. Using the DBT framework, describe your approach to treatment prioritisation.

02

Clinical Quick Reference

Disorder-Specific Therapy Map — Weave Psychotherapy
Vol. 11

D6: Disorder-Specific Therapy Map — Quick Reference

1. MASTER DISORDER-THERAPY TABLE

The single most important table in this series. Covers 32 disorder-therapy pairings with guideline sources.

#	Disorder	1st Line Therapy	2nd Line	Emerging / Adjunct	NICE Guideline	Key Trial
1	MDD (mild-moderate)	CBT, BA, IPT	Guided self-help, STPP	MBCT (relapse prevention)	CG90/NG222	Rush 1977; Elkin 1989
2	MDD (severe)	CBT + SSRI, IPT + SSRI	BA + SSRI	CBASP	CG90/NG222	Cuijpers 2020
3	Persistent Depressive Disorder	CBT, IPT	CBASP, Schema Therapy	MBCT	(as MDD)	Keller 2000 (NEJM)
4	Treatment-Resistant Depression	CBT augmentation of medication	STPP, Schema Therapy	Ketamine-assisted therapy	CG90	Wiles 2013 (CoBalT)
5	GAD	CBT, Applied Relaxation	STPP, mindfulness	Metacognitive Therapy (Wells)	CG113	Borkovec 1995
6	Social Anxiety Disorder	CBT (Clark & Wells model)	STPP (Leichsenring 2013)	Internet-delivered CBT	CG159	Clark 2006
7	Specific Phobia	In vivo graded exposure	Systematic desensitisation	Virtual reality exposure	—	Ost 1989
8	Panic Disorder	CBT (Clark model)	Applied Relaxation	Internet-delivered CBT	CG113	Clark 1994, 1999

#	Disorder	1st Line Therapy	2nd Line	Emerging / Adjunct	NICE Guideline	Key Trial
9	Agoraphobia	CBT + graded in vivo exposure	Guided self-help exposure	VR exposure	CG113	—
10	OCD	CBT with ERP (Salkovskis model)	SSRI (high dose)	Intensive residential ERP	CG31	Foa 2005
11	BDD	CBT (adapted for BDD)	SSRI	—	CG31	Veale 2010
12	Hoarding Disorder	CBT (adapted) + MI	SSRI	—	—	Steketee 2007
13	PTSD	TF-CBT (PE, CPT), EMDR	SSRI (sertraline, paroxetine)	Imagery rescripting	NG116	Powers 2010; Bisson 2007
14	Complex PTSD	Phase-based: STAIR + PE	Schema Therapy, EMDR + stabilisation	—	(NG116)	Cloitre 2010
15	Adjustment Disorder	Supportive counselling, brief CBT	Problem-solving therapy	—	—	Cuijpers 2020
16	BPD	DBT, MBT	TFP, Schema Therapy	Group Schema Therapy	CG78	Linehan 2006; Bateman 2008

#	Disorder	1st Line Therapy	2nd Line	Emerging / Adjunct	NICE Guideline	Key Trial
17	ASPD	Forensic CBT programmes	MI + anger management	—	CG77	Woody 1985
18	NPD	Schema Therapy	Psychodynamic (Kohut/Kernberg)	CT-PD (Beck)	—	—
19	AvPD	CBT, Schema Therapy	STPP	Group CBT	—	—
20	OCPD	CBT (Beck CT-PD)	Schema Therapy	Psychodynamic	—	—
21	Dependent PD	CBT (graded autonomy), Schema Therapy	Psychodynamic	—	—	—
22	AN (adolescent)	FBT (Maudsley model)	Individual therapy	—	NG69	Lock 2010
23	AN (adult)	CBT-ED, MANTRA, SSCM	Focal psychodynamic, ST	—	NG69	Zipfel 2014 (ANTOP)
24	BN	CBT-E (Fairburn)	IPT	Guided self-help	NG69	Fairburn 2009
25	BED	CBT, guided self-help CBT	IPT	—	NG69	Wilfley 2002

#	Disorder	1st Line Therapy	2nd Line	Emerging / Adjunct	NICE Guideline	Key Trial
26	Alcohol Use Disorder	MI, CBT relapse prevention	CRA, TSF (12-step facilitation)	Contingency management	CG115	Project MATCH 1997
27	Opioid Use Disorder	CBT relapse prevention + OAT	Contingency management	CRA	CG51	—
28	Stimulant Use Disorder	CBT, Contingency management	CRA, MI	—	—	—
29	Schizophrenia	CBTp + family intervention	Social skills training	CT-R (Beck)	CG178	Morrison 2014; Pilling 2002
30	Bipolar Disorder	Psychoeducation + mood stabiliser	IPSRT, CBT, FFT	—	CG185	Colom 2003; Frank 2005
31	ADHD (child)	Parent training, school BT	CBT skills (adjunct)	—	NG87	MTA Study 1999
32	Conduct Disorder	Parent training (child); MST (adolescent)	PCIT, FFT	—	CG158	Henggeler 1998
33	Adolescent Depression	CBT + SSRI (combined), IPT-A	CBT alone, fluoxetine alone	DBT-A (with self-harm)	NG134	TADS 2004

#	Disorder	1st Line Therapy	2nd Line	Emerging / Adjunct	NICE Guideline	Key Trial
34	Separation Anxiety (child)	CBT (Coping Cat programme)	Family CBT	—	—	Kendall 1997
35	Adolescent AN	FBT (Maudsley)	Individual therapy	—	NG69	Lock 2010
36	Adolescent Self-Harm	DBT-A (Rathus & Miller)	MBT-A	—	—	Mehlum 2014
37	Couples Distress	EFT-C (Johnson), Gottman Method	BCT, IBCT	—	—	Johnson 1999

2. BPD TREATMENTS HEAD-TO-HEAD

Feature	DBT	MBT	TFP	Schema Therapy
Developer	Linehan	Bateman & Fonagy	Kernberg	Young
Model	Biosocial (biology + invalidation)	Attachment (impaired mentalisation)	Object relations (identity diffusion)	Schema modes (unmet childhood needs)
Primary target	Emotion dysregulation, self-harm	Mentalisation capacity	Identity integration, affect modulation	Schema healing, mode change
Therapist stance	Acceptance + change (dialectical)	Curious, not-knowing	Neutral, interpretive	Limited reparenting + empathic confrontation
Structure	Individual + skills group + phone coaching + consult team	Individual + group analytic (1×/wk each)	Individual (2×/wk)	Individual (1–2×/wk)
Duration	1 year	18 months + 18 months stepdown	1–3 years	2–3 years
Key techniques	Chain analysis, diary cards, 4 skills modules, validation	Clarification, elaboration, challenge, mentalising transference	Transference interpretation, confrontation	Imagery rescripting, chair work, mode dialogues, flash cards
Key trial	Linehan 2006	Bateman & Fonagy 2008	Doering 2010; Clarkin 2007	Giesen-Bley 2006

Feature	DBT	MBT	TFP	Schema Therapy
Recovery rate	Significant reduction in self-harm	Improved social function, reduced self-harm	Improved personality organisation	45% recovery (vs. 24% TFP)
Dropout rate	~25%	~25%	~50%	~27%
NICE recommended	Yes	Yes	No	No
Best for	High-risk, actively suicidal	Comorbid depression, low mentalisation	Higher-functioning, identity confusion	Chronic BPD, CBT non-responders

EXAM PEARL

Giesen-Bley 2006 is the only head-to-head RCT showing superiority: ST > TFP (recovery 45% vs. 24%, dropout 27% vs. 50%).

EXAM PEARL

DBT target hierarchy (governs every session): (1) Life-threatening behaviours → (2) Therapy-interfering behaviours → (3) Quality-of-life-interfering behaviours → (4) Increasing skills.

3. DEPRESSION THERAPIES RANKED

Therapy	Effect Size (d)	Best For	NNT	Key Limitation
BA	0.74	Severe depression; resource-limited settings	~5	Less relapse prevention evidence
CBT	0.71	Mild-severe; most evidence overall	~5	May be less effective in severe depression as monotherapy
STPP	0.69	Characterological depression	~5	Fewer RCTs; longer training
IPT	0.67	Depression with interpersonal problems	~5	Limited therapist availability
MBCT	RR 0.69 (relapse)	Relapse prevention (3+ episodes)	~4	NOT for acute depression
CBASP	—	Chronic/persistent depression	—	Complex; limited evidence

EXAM PEARL

For mild-moderate depression, no therapy is clearly superior to another — the Dodo bird verdict applies (Wampold, 2015). For severe depression, combination (therapy + medication) is consistently superior to either alone.

CLINICAL ANCHOR

MANAS trial (Patel 2010, Lancet): BA delivered by lay health workers was effective for common mental disorders in India — major implication for low-resource settings.

4. ANXIETY THERAPIES BY DISORDER

Anxiety Disorder	1st Line Protocol	Key CBT Model	Unique Technique	Effect Size
GAD	CBT (16–20 sessions)	Borkovec; Dugas (IU); Wells (metacognitive)	Worry postponement, behavioural experiments for IU	d = 0.80
Social Anxiety	CBT (Clark & Wells)	Self-focused attention + safety behaviours + post-event rumination	Video feedback, attention training, behavioural experiments	d = 1.20
Specific Phobia	In vivo exposure (1–5 sessions)	Habituation / inhibitory learning	Single-session exposure (Ost 1989); applied tension for BII phobia	d = 1.05
Panic Disorder	CBT (Clark model)	Catastrophic misinterpretation of bodily sensations	Interoceptive exposure (hyperventilation, spinning)	d = 0.90
Agoraphobia	CBT + graded exposure	Avoidance maintains fear	Systematic in vivo exposure hierarchy	d = 0.80

EXAM PEARL

Applied tension (Ost & Sterner 1987) is specific to blood-injection-injury phobia — the ONLY phobia subtype with a parasympathetic (vasovagal) rather than sympathetic response pattern.

EXAM PEARL

Clark & Wells model for social anxiety has three maintaining factors: (1) self-focused attention, (2) safety behaviours, (3) anticipatory and post-event processing. Most examined CBT anxiety model.

5. EVIDENCE LEVELS — QUICK GUIDE

NICE Recommendation Strength

Term	Meaning
"Offer"	Strong recommendation — do this
"Consider"	Conditional recommendation — weigh individual factors
"Do not routinely offer"	Evidence does not support routine use
"Do not offer"	Evidence of harm or no benefit

Effect Size Interpretation

Cohen's d	NNT (approx.)	Label
0.2	~16	Small
0.5	~6	Medium
0.8	~4	Large
1.0+	~3	Very large

Key Meta-Analysts to Know

Researcher	Contribution
Cuijpers P	Depression psychotherapy meta-analyses (VU Amsterdam); most cited researcher in the field
Leichsenring F	Psychodynamic therapy meta-analyses; head-to-head comparisons with CBT
Shedler J	"Where is the evidence for evidence-based therapy?" (2010) — case for psychodynamic therapy
Wampold BE	Common factors model; Dodo bird verdict; alliance accounts for more variance than specific techniques
Jauhar S	CBTp meta-analyses (smaller effects than earlier reviews)

6. KEY THERAPY ACRONYMS

Acronym	Full Name	Used For
CBT	Cognitive Behaviour Therapy	Depression, anxiety, OCD, PTSD, psychosis
BA	Behavioural Activation	Depression
IPT	Interpersonal Therapy	Depression, BN, BED
MBCT	Mindfulness-Based Cognitive Therapy	Depression relapse prevention
CBASP	Cognitive Behavioural Analysis System of Psychotherapy	Chronic depression
ERP	Exposure and Response Prevention	OCD
PE	Prolonged Exposure	PTSD
CPT	Cognitive Processing Therapy	PTSD
EMDR	Eye Movement Desensitisation and Reprocessing	PTSD
STAIR	Skills Training in Affective and Interpersonal Regulation	Complex PTSD (Phase 1)
DBT	Dialectical Behaviour Therapy	BPD, self-harm, emotion dysregulation
MBT	Mentalisation-Based Treatment	BPD
TFP	Transference-Focused Psychotherapy	BPD
ST	Schema Therapy	BPD, NPD, chronic PDs

Acronym	Full Name	Used For
CT-PD	Cognitive Therapy of Personality Disorders	All PDs (Beck framework)
CBT-E	Enhanced CBT	Eating disorders (transdiagnostic)
FBT	Family-Based Treatment (Maudsley)	Adolescent AN
MANTRA	Maudsley AN Treatment for Adults	Adult AN
SSCM	Specialist Supportive Clinical Management	Adult AN
MI	Motivational Interviewing	Substance use, ambivalence
CRA	Community Reinforcement Approach	Alcohol use disorder
TSF	Twelve-Step Facilitation	Alcohol use disorder
CBTp	CBT for Psychosis	Schizophrenia
CT-R	Recovery-Oriented Cognitive Therapy	Schizophrenia
IPSRT	Interpersonal and Social Rhythm Therapy	Bipolar disorder
FFT	Family-Focused Therapy	Bipolar disorder, conduct disorder
MST	Multisystemic Therapy	Conduct disorder (adolescent)
PCIT	Parent-Child Interaction Therapy	Conduct disorder (child)
EFT-C	Emotionally Focused Therapy for Couples	Couples distress

Acronym	Full Name	Used For
IBCT	Integrative Behavioural Couples Therapy	Couples distress

7. VIVA QUESTIONS (12)

1. **"Talk me through the NICE stepped-care model for depression."** Start with watchful waiting/guided self-help (mild), through BA/CBT/IPT (moderate), to combined therapy + medication (severe), to specialist referral (treatment-resistant). Cite CG90/NG222.

1. **"What is the evidence for CBT vs. medication in OCD?"** Foa 2005: ERP = clomipramine; ERP alone is sufficient for moderate OCD. NICE recommends ERP as first-line for mild-moderate; SSRI + ERP for moderate-severe.

1. **"Describe the four evidence-based treatments for BPD."** DBT (biosocial, emotion dysregulation), MBT (attachment, mentalisation), TFP (object relations, identity), ST (schemas, mode work). Giesen-Bley 2006 is the key head-to-head trial.

1. **"A patient has recurrent depression with 4 prior episodes. What relapse prevention strategy would you recommend?"** MBCT (Segal, Williams, Teasdale). NNT = 4 for preventing relapse with 3+ episodes. Kuyken 2016. 8–week group programme. NOT for acute depression.

1. **"Compare PE and EMDR for PTSD."** Both are NICE first-line (NG116). PE has more RCT evidence; EMDR has comparable outcomes (Bisson 2007 Cochrane). PE uses imaginal + in vivo exposure; EMDR uses bilateral stimulation + trauma processing. Mechanism of EMDR is debated (working memory taxation vs. exposure component).

1. **"What is the role of psychological therapy in bipolar disorder?"** Adjunct to mood stabilisers, never standalone. Hierarchy: psychoeducation (Colom

2003), IPSRT (Frank 2005), CBT (Lam 2003 — effective only if <12 prior episodes), FFT (Miklowitz 2003).

1. **"Describe the Clark model for panic disorder."** Bodily sensations → catastrophic misinterpretation → increased anxiety → increased sensations (positive feedback loop). Treatment: cognitive restructuring + interoceptive exposure + dropping safety behaviours. 80–90% panic-free rates.

1. **"How does CBT for personality disorders differ from standard CBT?"** Beck CT-PD: longer duration, targets schemas not just automatic thoughts, therapeutic relationship needs more attention, experiential techniques essential (not optional), resistance as data, three schema operations (restructuring/modification/reinterpretation).

1. **"What therapy would you recommend for adolescent anorexia nervosa?"** FBT (Maudsley model) — Lock & Le Grange. Three phases: parents take control of feeding, gradual return of control to adolescent, developmental/identity issues. Agnostic about aetiology. NICE first-line (NG69).

1. **"A 15-year-old with BPD features and recurrent self-harm is referred. What psychological treatment?"** DBT-A (Rathus & Miller). Key differences from adult DBT: 5th module (Walking the Middle Path), multifamily skills group, 24-week duration. Mehlum 2014 RCT showed significant reduction in self-harm.

1. **"What is motivational interviewing and when is it used?"** MI (Miller & Rollnick) is a communication style, not a full therapy. Spirit: partnership, acceptance, compassion, evocation. Four processes: engaging, focusing, evoking, planning. Resolves ambivalence by eliciting change talk. Used across substance use disorders and as a prelude to behaviour change in any condition.

1. **"What is the Dodo bird verdict and what does it mean for clinical practice?"** Rosenzweig (1936), confirmed by Wampold (2015): when bona fide therapies are compared head-to-head, differences in outcomes are small and often non-significant. Common factors (alliance, empathy, expectations) account

for more outcome variance than specific techniques. Clinical implication: therapist effects may be larger than therapy brand effects. Counter-argument: some disorders have clear treatment-of-choice (ERP for OCD, exposure for phobias).

8. LANDMARK TRIALS — QUICK INDEX

Trial	Year	Disorder	Finding
Rush et al.	1977	Depression	First RCT: CBT = imipramine
Elkin et al. (NIMH TDCRP)	1989	Depression	CBT = IPT = imipramine (mild-moderate); imipramine superior in severe
Dimidjian et al.	2006	Severe depression	BA = antidepressant > CBT (acute phase)
Kuyken et al.	2016	Recurrent depression	MBCT reduces relapse; NNT = 4 for 3+ episodes
Wiles et al. (CoBalT)	2013	Treatment-resistant depression	CBT augmentation effective; NNT = 4
Keller et al.	2000	Chronic depression	CBASP + nefazodone > either alone (NEJM)
Patel et al. (MANAS)	2010	Common mental disorders	Lay-delivered BA effective in India (Lancet)
Clark et al.	1994, 2006	Panic, Social anxiety	CBT 80–90% panic-free; CBT > fluoxetine for social anxiety
Foa et al.	2005	OCD	ERP = clomipramine; combination not clearly superior
Bisson et al. (Cochrane)	2007	PTSD	TF-CBT and EMDR both effective; both first-line

Trial	Year	Disorder	Finding
Cloitre et al.	2010	Complex PTSD	STAIR + PE effective (phase-based approach)
Linehan et al.	1991, 2006	BPD	DBT reduced self-harm and hospitalisation
Bateman & Fonagy	1999, 2008	BPD	MBT reduced self-harm, improved social function; gains maintained 8 years
Giesen-Bley et al.	2006	BPD	ST (45% recovery) > TFP (24% recovery); lower dropout
Fairburn et al.	2009	BN	CBT-E: ~50% remission
Lock et al.	2010	Adolescent AN	FBT (Maudsley): sustained weight restoration at 4 years
Project MATCH	1997	Alcohol dependence	TSF = CBT = MET at 1–year and 3–year follow-up
Colom et al.	2003, 2009	Bipolar disorder	Group psychoeducation reduced relapse at 5–year follow-up
Frank et al.	2005	Bipolar disorder	IPSRT lengthened time to recurrence

Trial	Year	Disorder	Finding
MTA Study	1999	ADHD (child)	Medication > behavioural treatment for core symptoms; combined best for associated problems
TADS	2004	Adolescent depression	CBT + fluoxetine (71%) > fluoxetine (61%) > CBT (43%) > placebo (35%)
Mehlum et al.	2014	Adolescent self-harm	DBT-A reduced self-harm and suicidality
Morrison et al.	2014	Schizophrenia	CBTp: d = 0.33–0.44 vs. TAU
Pilling et al.	2002	Schizophrenia	Family intervention reduced relapse by ~50%
Johnson et al.	1999	Couples distress	EFT-C: d = 1.31; 70–75% recovery
Kendall et al.	1997	Child anxiety	CBT (Coping Cat): 71% no longer met diagnosis

9. FIVE THINGS TO REMEMBER

1. **Depression:** CBT = IPT = BA for mild-moderate. Combination is superior for severe. MBCT for relapse prevention (3+ episodes).
2. **Anxiety:** Each disorder has a specific CBT model (Clark & Wells for social anxiety, Clark for panic, Salkovskis for OCD, Borkovec/Dugas for GAD). Know the model, not just "CBT."

3. **BPD:** Four treatments (DBT, MBT, TFP, ST). NICE recommends DBT and MBT. Giesen-Bley 2006 showed ST > TFP. DBT target hierarchy is heavily examined.
4. **Psychosis:** CBTp is an adjunct (small effect, d = 0.33–0.44, but clinically meaningful). Family intervention reduces relapse by 50% (Pilling 2002).
5. **Children:** FBT for adolescent AN, parent training for ADHD/conduct disorder, CBT + SSRI for adolescent depression (TADS 2004), DBT-A for adolescent self-harm (Mehlum 2014).

WEAVE

CENTRE FOR INTEGRATIVE PSYCHIATRY

Psychiatry that listens before it prescribes.

WEBSITE

www.weave.clinic

INSTAGRAM

@weave.clinic · @doc.wilfred.md · @doc.niharika.md

EMAIL

wilfred.desouza1996@gmail.com

Two doctors. Two approaches. One practice.

ADHD · Relationships · Mood · Addiction · Children

Online across India